

Surgery for advanced prostate cancer

Some people have an operation to help with the symptoms of advanced prostate cancer. Find out about the different types of surgery that may be used.

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About surgery for advanced prostate cancer

Surgery can be used to help control [advanced \(metastatic\) prostate cancer](#) and relieve symptoms. Surgery can be used:

- instead of hormonal therapy, to reduce testosterone
- if the cancer is making it difficult to pass urine (pee)
- to strengthen a bone.

Surgery to remove the prostate gland (radical prostatectomy) is not used to treat advanced prostate cancer. This is because a radical prostatectomy cannot cure prostate cancer once the cancer cells have spread outside the prostate gland and nearby area.

Surgery to reduce testosterone

Some people may have surgery, instead of [hormonal therapy](#), to reduce the amount of testosterone the body produces naturally. The operation involves removing part of the testicles. This is called a subcapsular orchidectomy.

This surgery is not reversible so the hormonal effects are permanent. It is rarely done now, as most people have hormonal therapy to reduce testosterone instead.

Surgery for problems with passing urine

Surgery can be used to help if the cancer is affecting the flow of urine through your:

- ureters – the tubes that drain urine from the kidneys into the bladder
- urethra – the tube that you pass urine out of the body from.

Surgery for prostate cancer that is affecting the urethra is called a [transurethral resection of the prostate \(TURP\)](#). Surgery for prostate cancer that is affecting the ureters will either be a percutaneous nephrostomy or JJ stent.

Transurethral resection of the prostate (TURP)

Your surgeon may offer you a transurethral resection of the prostate (TURP) if you are having difficulty passing urine (peeing). A TURP is not done to treat the cancer.

The aim of the surgery is to remove the area of prostate that is pressing on the [urethra](#). It can help you pass urine more easily again and improve symptoms.

You may have a TURP before radiotherapy to the prostate. This is because radiotherapy can make passing urine more difficult during treatment and for a while afterwards.

We have [more information about having a TURP](#).

Percutaneous nephrostomy or JJ stent

The tubes that drain urine from the kidneys to the bladder (ureters) may get blocked by prostate cancer. This will make it difficult to pass urine (pee). If this happens, you may be offered 1 of the following types of surgery:

- [Percutaneous nephrostomy](#) – the doctor, guided by a scan, puts a tube into the kidney. The tube goes into a bag that sits outside on the skin. Urine can then drain into the bag.
- JJ stent, also called a [ureteric stent](#) – this is when the doctor puts a fine tube into the ureter. This opens up the ureter so urine can flow through it.

A planned percutaneous nephrostomy can often be done under a local anaesthetic with a sedative (a drug to make you relaxed and sleepy). You usually do not need to stay in the hospital overnight.

Your cancer team can give you more information about both these types of surgery.

Surgery to strengthen a weakened bone

Prostate cancer that has spread to the bones may cause problems that need treating with surgery.

If you are at risk of a broken bone, your doctor may advise [surgery to repair or strengthen the bone](#) to help prevent it from breaking in the future.

Going home after surgery

Before you leave hospital after surgery for advanced prostate cancer, your hospital doctor will send a letter to your GP. This is called a discharge summary. It is usually sent electronically so that your GP gets it within 24 hours of your discharge. You are usually given a copy to take home with you. It will explain what type of surgery you had, any medication you are taking and about your follow-up care.

If you need a district or community nurse, the ward nurses will organise this before you go home. You should also be given 24-hour telephone numbers for your hospital team in case you need to contact them.

Your surgeon or specialist nurse can give you advice on when you can start doing things such as driving or returning to work.

Macmillan is also here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line on [0808 808 00 00](tel:0808 808 00 00).
- [Chat to our specialists online](#).
- Visit our [prostate cancer forum](#) to connect with people who have been affected by prostate cancer, share your experience, and ask your questions.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our prostate cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Prostate cancer: diagnosis and management. NICE guideline [NG131] Published: 09 May 2019 Last updated: 15 December 2021 (accessed October 2024) <https://www.nice.org.uk/guidance/ng131>

Parker, C. et al. Prostate cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Annals of Oncology, Volume 31, Issue 9, 1119–1134 (accessed October 2024) [https://www.annalsofoncology.org/article/S0923-7534\(20\)39898-7/fulltext#articleInformation](https://www.annalsofoncology.org/article/S0923-7534(20)39898-7/fulltext#articleInformation)

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

An anonymous network of people affected by prostate cancer which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help